



UniSports Ortho Ltd

Anaesthetic ACC Policy – 1st October 2026 to 30th June 2027

Pricing for all cases (including Red List and Non- Core) has increased by 0.77% and is in line with ACC's increase.

The new fee per unit is \$93.47 excluding gst or **\$107.49 including gst.**

Other than the price, there is no change to the guidelines. Please communicate this document to your anaesthetists.

In general, we try to align with ACC's operational guidelines particularly for modifiers. As we often need to provide documented proof, **all invoices need to clearly itemise the reason/s modifier units are being claimed.**

These guidelines will be automatically applied to invoices received. Your administrative staff are welcome to ask about payment changes within a reasonable time frame.

Many thanks
Ashley Furlong
Business Manager
Unisports Ortho Ltd
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RVG and Guidelines Introduction

This set of guidelines reflects some of the RVG mechanisms for generating anaesthetic fees for Unisports ACC contracted cases. Unisports acknowledges there may be some differences between these guidelines and the 2021 RVG.

Unisports believes the unit rate offered to anaesthetists for Unisports contracted cases is an appropriate framework and pay using (only) these guidelines.

At all times Unisports acknowledges on rare occasions extra work for any case may not be fully compensated and if this situation occurs, please discuss with the surgeon at the time and communicate the details to Ashley Furlong.

BASE UNITS

As per 2021 RVG base units are categorized according to the anatomical site of the procedure being performed and complexity of the anaesthesia required. Base units consider the anticipated physiological stressors of the given procedure. Patient complexity separate to the surgical procedure comes under modifying units.

Base units also take into account a brief (<15 minutes) pre and post-operative visit.

When multiple procedures are performed during one anaesthetic only ONE base unit should be charged. If a second procedure requires a change of position, modifying units may be added.

TIME UNITS

Time units are in accordance with the OR times we are charged for. **Time units will be checked against the time recorded on the hospital invoice and automatically paid accordingly.** The RVG allocation will be 1 time unit per 15 minutes until 120 minutes has passed and then one time unit per 10 minutes after that.

Pre-op visits and phone calls

ACC has agreed to pay for some of this work for ACC patients and can be charged as part of a clinical services contract. If your organisation does not have a clinical services contract, the Auckland Surgical Centre is willing to be the processor for these fees.

Please get in touch with your anaesthetic group administrator to make sure you are following the ACC guidelines. At all times Unisports is willing to provide letters of referral to expedite and assist the anaesthetists as they work the ACC process to obtain ACC pre-op assessment fees.

Pre-OR Time

Apart from insertion of a block outside the OR time will start when the patient enters the OR. **Time units will be checked against the OR times we receive from the hospital and paid accordingly.**



PACU time

Usually there is a brief handover to the PACU nurses with this time being part of the base units. If you have a more complicated handover for any reason, please clearly explain this on an invoice and charge as a modifier in accordance with the guidelines below.

MODIFIERS

Unisports will pay in accordance with ACC's operational guidelines. Any modifier claimed must be clearly explained on an invoice.

- Age if > 1 and/but <2, 70-80 year old = 1 unit
- Age if < 1 or > 80 = 2 units
- ASA1 = 0 units
- ASA2 = 0 units
- ASA3 = 2 units
- ASA4 = 4 units
- Awake Intubation (however accomplished) = 2 units
- Positioning - Beach chair positioning = 2 units (no claim for lateral)
- Prone positioning = 2 unit
- BMI 35 – 40 = 1 unit
- BMI >40 = 2 units
- BMI >50 = 3 units
- Expected arterial line, CVL, PA catheter insertion = 0 units
- Unexpected arterial line, CVL PA catheter insertion = 2 units
- HDU – (with the anaesthetist at home providing cover):
 - Base pricing recognizing availability = 4 units
 - Telephone calls which require significant management change = 1 unit per call
 - Any face-to-face visit that results in significant management change = 4 units per visit plus time
- Ward care - prolonged PACU case is shown as modifiers
 - Visits to the PACU to review case SIMPLE = 2 units per visit, with a maximum of 2 visits per day (non-complex involving epidurals, IV fluids, simple pain management)
 - Visits to the PACU to review case COMPLEX (longer than 15 minutes) = Up to 2 units (plus up to 2 more units with justification), complex visits with significant documented management issues.
 - Prolonged PACU care – time units shown as modifiers.
- Continuous regional block. Plexus nerve catheter care (which stay in the patient overnight or for several days) = up to a maximum of 3 units
- Blocks - If a block is done outside the OR then UniSports will pay an additional 1 unit for this.
- Abandoned Surgery – modifier costs – patient found not to be fit for surgery after admission, patient found to have not fasted after admission, patient changed their mind after admission.
- Cancellation of case on day of surgery after assessment and management of the relevant issues = 2 units. This includes patients found to be not fit for surgery after admission, patients found to have not fasted after admission, patient changed their mind after admission.